

CHAROTAR UNIVERSITY OF SCIENCE AND TECHNOLOGY

Third Semester M.Sc Nursing Examination

April-2018

Course Code & Name: NR 831 OBSTETRIC AND GYNAECOLOGICAL NURSING-III

Date & Day: 11.04.2018, Wednesday

Time: 10:00 a.m. to 10:30 a.m.

Maximum Marks: 15

SECTION-I

Q.1. Multiple choice question: [Tick ✓ correct answer in given bracket] 15x1=15

1. The condition of Hydramnios is not associated with
 - a. Renal agenesis []
 - b. Diabetes insipidus []
 - c. Polycystic kidney []
 - d. Multiple pregnancy []
2. Lovset's manuevure is done for
 - a. Delivery of after coming head []
 - b. Breech extraction []
 - c. Delivery of the arms and shoulder []
 - d. Delivery of the placenta []
3. is the Antihypertensive drug which inhibits labour.
 - a. Reserpine []
 - b. Diazoxide []
 - c. Minoxidil []
 - d. Captopril []
4. Bishop scoring include assessment of all of the following except
 - a. Dilatation of cervix []
 - b. Effacement of cervix []
 - c. Uterine contraction []
 - d. Station of the head []
5. Medical treatment of ectopic gestation involves use of all of the following except
 - a. Prostaglandins []
 - b. Methotrexate []
 - c. RU-486 []
 - d. Dexamethasone []
6. Large placenta is seen in
 - a. IUGR []
 - b. Syphilis []
 - c. Rubella []
 - d. CMV []
7. DIC is seen in
 - a. Retained placenta []
 - b. Abruptio placenta []
 - c. Intra uterine death of fetus []
 - d. All the above []
8. is associated with hydatidiform mole.
 - a. Follicular cyst []
 - b. Theca lutein cyst []
 - c. Ovarian adenoma []
 - d. Pseudocyst ovary []

9. PPTCT intervention package includes all of the following except
 a. Antenatal care & Institutional delivery []
 b. Counseling []
 c. Tetanus toxoid immunization []
 d. Rapid HIV testing []
10. Vacuum extraction is done when
 a. Cervix is dilated more than 8 cm []
 b. Persistent occipito posterior position is present []
 c. Deep transverse arrest is noted []
 d. Acute maternal distress occurs in second stage of labour []
11. The condition of feeling sad or moody for a short period that is triggered by hormonal changes after giving birth is called.....
 a. Post partum blues []
 b. Post partum depression []
 c. Post partum psychosis []
 d. None of the above []
12.babies are those whose birth weight is below tenth percentile of the average of gestational age.
 a. SGA []
 b. LGA []
 c. AGA []
 d. IUGR []
13. regarding HELLP syndrome is true.
 a. May occur in non-pregnant patients []
 b. Abdominal pain is characteristic feature []
 c. Related to pre-eclampsia []
 d. Patients always have tar coloured vomitting []
14. is not a risk factor for ectopic pregnancy.
 a. Fertility treatment []
 b. Family history []
 c. Endometriosis []
 d. Previous tubal infection []
15. Pathological contraction ring is due to.....
 a. Obstructed labour []
 b. Uterine inertia []
 c. Preterm labor []
 d. Oligohydramnios []

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Date & Day: 11.04.2018, Wednesday Time: 10:30 a.m. to 1:00 p.m. Maximum Marks: 60

Instructions:

- *Write each section in separate booklet.*
- *Draw neat labelled diagrams wherever necessary.*

SECTION-II

- Q.2.** a. Define Asphyxia neonatorum. **[2+3+5]**
 b. Enumerate the risk factors associated with Asphyxia neonatorum.
 c. Explain the specific management of Asphyxia neonatorum.
- Q.3.** a. Write about Inversion of Uterus. **[5+5]**
 b. Write about pregnancies at risk-due to pre-existing Cardio-vascular disease.

OR

- Q.3.** Write in detail about puerperal sepsis. **[10]**
- Q.4.** Answer **Any Two** of the following: **[2x5]**
 a. Breast complications during puerperium.
 b. Management of Anemia during pregnancy.
 c. TORCH infections.

SECTION-III

- Q.5.** a. Describe the sign & symptoms of Pre-eclampsia. **[3+7]**
 b. Write down the management of Eclampsia
- Q.6.** a. Define APH. **[1+2+7]**
 b. Enumerate the causes of APH.
 c. Write down in detail about Placenta Praevia.

OR

- Q.6.** a. Describe the causes of PPH. **[4+6]**
 b. Write down the management of true PPH.
- Q.7.** Answer **Any Two** of the following: **[2x5]**
 a. Prolonged labour
 b. Oligohydramnios
 c. Gestational diabetes

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Maximum Marks: 75

ANSWER KEY

SECTION-I

Q.1. Multiple choice question

15x1=15

1. a
2. c
3. b
4. c
5. d
6. c
7. d
8. b
9. c
10. d
11. a
12. a
13. c
14. b
15. a

SECTION-II

- Q.2**
- a. Asphyxia neonatorum: A condition of impaired blood gas exchange, that if persists leads to progressive hypoxaemia, hypercapnia & metabolic acidosis.
 - b. Risk factors: Maternal and neonatal/fetal risk factors.
 - c. Management including basic and definitive management. APGAR, Neonatal resuscitation, medications, intubation, PPV
- Q.3**
- a. Inversion of Uterus: Definition, types/degree, causes, clinical features, diagnosis, management, complication.
 - b. Pregnancies at risk-due to pre-existing Cardio-vascular disease: meaning, causes, types of cardio vascular diseases, effect of these diseases on various body systems during pregnancy, signs and symptoms, diagnosis and management.

OR

- Q.3** Puerperal sepsis: definition, causative organism, risk factor, mode of infection, degree/classification, signs and symptoms, diagnostic evaluation, management, complication
- Q.4**
- Breast complications during puerperium: Breast infection/mastitis and abscess.
 - Management of Anemia during pregnancy: Prophylactic management and Specific treatment- Oral therapy, Parenteral therapy, Blood transfusion, exchange transfusion, Management during labour & puerperium
 - TORCH infections: Toxoplasmosis, Other, Rubella, Cytomegalo virus and Herpes simplex virus infection.

SECTION-III

- Q.5**
- Onset: insidious & slow, very rarely it may be acute.
 - Mild symptoms: Slight swelling over ankles which persists on rising from the bed in the morning or tightness of the finger ring. Gradually the swelling may extend to the face, abdominal wall, vulva & even the whole body.
 - Alarming symptoms: Headache-either occipital or frontal region, Disturbed sleep, Diminished urinary output: <400 ml/24 hr, Epigastric pain: associated with coffee color vomiting, Eye symptoms: blurring/dimness/ complete blindness
 - Signs: Abnormal weight gain, Rise in BP, Edema, Pulmonary edema (due to pulmonary congestion), Abdominal examination reveals placental insufficiency by scanty liquor or fetal growth retardation.
 - General management, Specific management and Obstetric management.
- Q.6**
- APH is bleeding from or into the genital tract after 28th week of pregnancy but before the birth of baby
 - Placental causes
Extra Placental causes
Unknown causes
 - Placenta Praevia: Definition, etiology, types, clinical features, diagnostic studies, complication and management.

OR

- Q.6**
- a.** Causes: Atonic, Traumatic, Mixed and Blood coagulopathy
 - b.** Management of True Post-Partum Haemorrhage: Surgical, Medical and nursing management. (schematic)
- Q.7**
- a.** Prolonged labour: definition, causes, signs and symptoms, diagnosis, management, complication/danger.
 - b.** Oligohydramnios : definition, causes, signs and symptoms, diagnosis, management.
 - c.** Gestational diabetes : definition, classification, risk factors, signs and symptoms, diagnosis, management, complication.

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SCHEME OF EVALUATION

SECTION-I

Q.1 One mark for each correct answer.

SECTION-II

- Q.2**
- a. One mark for each correct definition
 - b. One mark for each correct type
 - c. Five mark for correct management of abruptio placenta
- Q.3**
- a. Two mark for correct definition
 - b. One mark for each correct classification
 - c. Six marks for correct answer about inevitable abortion.

OR

Q.3 Two marks for each correct type of birth injuries to newborn.

Q.4 Five marks for each correct answer.

SECTION-III

- Q.5**
- a. Three mark for correct sign & symptoms of Pre-eclampsia
 - b. Seven mark for correct management of Eclampsia.
- Q.6**
- a. One mark for correct definition
 - b. Two mark for correct causes
 - c. Seven mark for correct answer about placenta praevia.

OR

- Q.6**
- a. Four mark for correct causes of PPH.
 - b. Six mark for correct management of true PPH.
- Q.7** Five marks for each correct answer.